

Anil Patel, MD — Breast Surgery, UCSF

1600 Divisadero St, 4th Floor — San Francisco, CA 94143 — (415) 555-0421

Patient: Rodriguez, Maria **DOB:** 03/14/1972 **MRN-UCSF:** 9912778

Date of Service: March 19, 2024 **Visit type:** New consultation

CHIEF COMPLAINT

Suspicious right breast mammogram finding, referred for surgical evaluation.

HPI

Pt seen in consultation for s/p suspicious mammogram. Reviewed images. Plan: schedule core needle biopsy for next week.

Maria is a 52-year-old peri-menopausal female referred by her OB/GYN (Dr. Wu) following a screening mammogram on 02/22/2024 that demonstrated a new spiculated mass in her right breast. Reviewing the mammogram report, the radiologist describes a suspicious finding at approximately the 1 o'clock position of the right breast, measuring 1.2 cm. She has been appropriately worried since being notified of the result. She has no breast pain, no skin changes, and no nipple discharge. She has not noted any palpable lump on self-exam since the result was reported.

PMH: peri-menopausal. PSH: none. Meds: none. NKDA. Social: lives with husband, two adult children, non-smoker, occasional wine.

Family hx: maternal aunt with breast cancer, age at diagnosis approximately 62. No first-degree affected. No known BRCA testing in the family.

EXAM

General: well-appearing, anxious-affect.

Breast exam: symmetric. Right breast: no skin changes, no nipple retraction. No mass palpable on focused exam at any position. No right axillary, supraclavicular, or infraclavicular lymphadenopathy. Left breast: unremarkable.

IMAGING REVIEW

Personally reviewed the 02/22/2024 mammogram on PACS. Confirmed a 1.2 cm spiculated mass in the right breast, in the **11 o'clock** position, approximately 3 cm from the nipple. (Note: the radiology impression and the present writer's reading agree on the 11 o'clock position; the HPI above inadvertently noted "1 o'clock" — this should be disregarded.)

ASSESSMENT & PLAN

1. **Suspicious right breast mass, BI-RADS 4 — needs tissue diagnosis.** Risk factors are limited (second-degree family hx only, peri-menopausal age, dense breast tissue). Differential includes fibroadenoma vs. invasive ductal carcinoma vs. other benign etiology.

Procedure: ultrasound-guided core needle biopsy of right breast 11 o'clock mass scheduled for 03/28/2024.

Risks of biopsy reviewed: bleeding, infection, bruising, false-negative. Patient understands and consents in principle; written consent will be obtained on the day of procedure. Will obtain ≥ 4 cores under US guidance. Pathology turnaround typically 3–5 business days.

2. **Patient education.** Spent 25 minutes counseling Maria on the diagnostic process, the range of possible outcomes, and the timeline for a definitive answer. She will receive results by phone within one week of the procedure and will return for in-person follow-up regardless of the result.

— Anil Patel, MD, FACS, Breast Surgery